

Policy on the Complete and Accurate Medical Record

«Hospital Name»

Document control

Document No./Version	_____ / 1.0	Issue No./Review due	01 / One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Medical Records In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» ensures every patient cared for has a complete and accurate medical record, uniquely identified, chronologically maintained, and entered only by authorised staff.

1. Purpose

This policy sets out how «Hospital Name» assigns a unique identifier to the medical record, identifies and documents its contents, keeps it complete and chronological, restricts entries to authorised staff, ensures entries are signed/dated/timed, keeps the author of each entry identifiable, and uses only authorised abbreviations.

This policy does not cover continuity-of-care content of the record, confidentiality and security of records, or document retention — those are covered in other hospital policies.

2. Scope

This policy applies to the medical record of every patient cared for at «Hospital Name», whether maintained physically, electronically, or as a mix of both.

3. Policy standards

«Hospital Name» assigns a unique identifier to every medical record.

«Hospital Name» identifies and documents the contents of the medical record.

«Hospital Name» ensures the medical record provides a complete, up-to-date and chronological account of patient care.

«Hospital Name» ensures only authorised staff make entries in the medical record.

«Hospital Name» ensures every entry in the medical record is signed, dated and timed.

«Hospital Name» ensures the author of every entry can be identified.

«Hospital Name» ensures the medical record has only authorised abbreviations.

4. Non-negotiable rules

1. Do not maintain a medical record without a unique identifier, on every sheet including digital media, traceable to that identifier.
2. Do not leave the contents of the medical record undefined — the documents that make up the record must be identified and adhered to.

3. Do not file a medical record out of chronological or sequential order, or leave a missing sheet without a note to that effect.
4. Do not let a staff member make an entry in the medical record without written authorisation defining who may enter what.
5. Do not leave an entry undocumented within one hour of completing the assessment or procedure, or unsigned, undated or untimed.
6. Do not leave the author of an entry untraceable — a name, employee code, stamp, or authorised e-signature must identify who made it.
7. Do not use an abbreviation in the medical record that is not on the organisation's standardised, approved list — and never an error-prone abbreviation for medications.
8. Staff who see a rule under this policy broken report it the same shift to the Medical Records In-Charge or the Medical Superintendent.

5. What we do

5.1 Unique identifier

Every medical record at «Hospital Name» has a unique identifier number. Every sheet in the medical record carries this identifier, including records on digital media. For electronic records, all entries for one unique identifier are available in one place and/or traceable to that number. Unique identification of the medical record is covered in other hospital policies as it relates to patient registration.

5.2 Contents of the medical record

«Hospital Name» defines the documents that form part of the medical record and adheres to that definition — for example admission orders, face sheet, IP sheet, referrals, transfer records, lab results, imaging results, discharge summary, doctor's order sheet, TPR chart, consent form, nursing assessment, nursing care plans, dietary assessments and plans, and physiotherapy assessments. Contents may be hand-written, typed, printed or electronic, or a mix, with appropriate linkages maintained between formats. Where electronic medical records are used, «Hospital Name» defines which documents are maintained in physical format.

5.3 Complete, chronological record

The medical record at «Hospital Name» has all identified sheets filed in sequential order, with entries in each component filed in chronological order. All medico-legal case records contain the mandatory information required. Where a sheet is missing, a note to that effect is placed in the medical record. Pages in the medical record are preferably numbered.

5.4 Authorised entries

«Hospital Name» has written guidance authorising who can make entries in the medical record and what content each authorised category may enter — for example medication orders by doctors, the medication administration chart by nurses, and nutritional assessment by dietitians. This authorisation is uniform across the organisation, even where different staff categories are authorised for different entry types.

5.5 Timing of entries

Entries in the medical record at «Hospital Name» are documented immediately, and no later than one hour after completion of the assessment or procedure. For records on electronic media, the date and time are preferably generated automatically by the system.

5.6 Identifying the author

The author of every entry in the medical record at «Hospital Name» is identifiable — by full name, employee code number, a stamp, or an authorised e-signature as per statutory requirements for electronic records. Traceability is achieved either by writing the name against every entry, by a master signature list held in the medical record, or by an employee code number against every entry, consistent with the authorisation record described in Section 5.4.

5.7 Authorised abbreviations

Where abbreviations are used in the medical record at «Hospital Name», a standardised list of approved abbreviations is used throughout the organisation. Error-prone abbreviations are not used for medications.

6. Governance and responsibility

Role	Responsibility
Medical Records In-Charge	Owns this policy. Maintains the unique-identifier system, contents definition, and abbreviation list.
Treating Doctors / Nurses / Allied Staff	Make entries strictly within their authorised category, signed, dated and timed.
Medical Superintendent	Oversees implementation of this policy. Receives escalations under Section 4.
All Staff	Use only authorised abbreviations. Report identified risks and rule breaches.

7. Quality monitoring

The Medical Records In-Charge reviews sampled medical records for unique identifier presence, chronological completeness, authorised entries, timely signing, author identifiability, and abbreviation compliance. Documentary evidence is on file for each CORE objective element and each asterisked objective element (marked CORE and * respectively in the Level column of Section 12, Traceability table). This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff who make entries in the medical record are trained on authorised entry categories, timing requirements, and the approved abbreviation list, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on the Complete and Accurate Medical Record of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the Medical Records department, all clinical department heads, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.3

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.3.a	CORE	Unique identifier assigned to the medical record	Section 5.1	Medical Records In-Charge
IMS.3.b	Commitment *	Contents of the medical record identified/documented	Section 5.2	Medical Records In-Charge
IMS.3.c	CORE	Complete, up-to-date, chronological record	Section 5.3	Medical Records In-Charge
IMS.3.d	Commitment *	Authorised staff make entries	Section 5.4	Medical Records In-Charge
IMS.3.e	Commitment	Entries signed, dated and timed	Section 5.5	Treating Doctors / Nurses / Allied Staff
IMS.3.f	Commitment	Author of the entry can be identified	Section 5.6 (see also Section 5.4)	Medical Records In-Charge
IMS.3.g	Commitment	Only authorised abbreviations used	Section 5.7	Medical Records In-Charge

12. Required Records / Evidence Checklist

IMS.3.a — Unique identifier.

- Unique-identifier assignment record for each patient's medical record.
- Traceability record confirming all entries for one identifier are locatable/linked.

IMS.3.b — Contents defined.

- Written definition of medical record contents, listing each document type.
- Format-mix linkage record where physical and electronic components co-exist.

IMS.3.c — Complete, chronological record.

- Sequential/chronological filing audit record for sampled files.
- Missing-sheet note record and medico-legal case mandatory-information check record.

IMS.3.d — Authorised entries.

- Written authorisation record naming which staff category may enter which content.
- Sampled-entry audit record confirming entries match the authorised category.

IMS.3.e — Timing of entries.

- Sampled-entry timing audit record confirming documentation within one hour of the assessment or procedure.
- System-generated timestamp record for electronic entries, where applicable.

IMS.3.f — Author identifiable.

- Master signature list, or employee-code/name traceability record, held in the medical record.
- E-signature authorisation record for electronic entries, per statutory requirements.

IMS.3.g — Authorised abbreviations.

- Standardised, approved abbreviations list, organisation-wide.
- Error-prone-abbreviation exclusion record for medication entries.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.3, "The patients cared for by the organisation have a complete and accurate medical record."

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

This document is not issued by, endorsed by, or affiliated with NABH or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.